

King Fahad Central Hospital

Stroke Alert (Code Stroke) Activation Policy

Emergency Department & In-Hospital Activation — Aligned with AHA/ASA Get With The Guidelines—Stroke and Joint Commission Stroke Center Certification Requirements

1. Purpose

This policy defines the criteria, mechanism, team roles, and response expectations for Stroke Alert ("Code Stroke") activation — for patients arriving with suspected acute stroke and for stroke symptom onset in already-hospitalized patients (in-hospital stroke) — to ensure rapid, guideline-concordant evaluation and treatment.

2. Activation Criteria

- ☐ Acute onset of focal neurologic deficit consistent with stroke (e.g., facial droop, arm/leg weakness, speech difficulty, visual change, ataxia, severe headache with neurologic deficit), regardless of symptom severity
- ☐ Symptom onset or last known well within a timeframe that could support acute intervention per institutional protocol (do not restrict activation solely by an arbitrary early cutoff — patients outside the typical thrombolysis window may still be thrombectomy or extended-window candidates)
- ☐ Positive prehospital (EMS) or ED triage stroke screen (e.g., Cincinnati Prehospital Stroke Scale, FAST-ED, or institutional equivalent)
- ☐ New focal neurologic deficit identified in an already-hospitalized patient, regardless of admitting diagnosis or unit
- ☐ Clinical suspicion of stroke maintained even with an atypical presentation or possible mimic — activation errs toward inclusion; stand-down occurs after evaluation, not before (see Section 6)

3. Who May Activate

- ☐ EMS personnel via pre-arrival notification to the receiving hospital (per institutional EMS–Hospital Stroke Collaboration Agreement)
- ☐ ED triage nurse or ED physician/APP upon patient arrival or triage assessment
- ☐ Any nurse, physician, APP, or other clinical staff member identifying a new focal neurologic deficit in a hospitalized patient (in-hospital stroke)
- ☐ Rapid response team, if called for a hospitalized patient found to have a new neurologic deficit

No activation is considered inappropriate for raising clinical concern — a rapid stand-down process (Section 6) is preferred over hesitation to activate.

4. Activation Mechanism

- ☐ Activator initiates Stroke Alert via institutional mechanism: _____ (e.g., overhead page "Stroke Alert, [location]", dedicated paging group, secure mobile alert application)
- ☐ Activation message includes: patient location, last known well/symptom onset time (if known), and brief symptom description
- ☐ Activation simultaneously notifies: stroke physician/neurology on call, ED physician (for ED activations), primary/charge nurse, CT technologist, pharmacy, laboratory, and stroke program coordinator per institutional distribution list
- ☐ For EMS pre-arrival notification, ED and CT team notified immediately upon receipt, prior to patient arrival, per institutional Acute Stroke CT Imaging Protocol activation workflow

5. Team Roles & Expected Response

Role	Expected Action / Response Time
Stroke physician / Neurology (or teleneurology)	Bedside evaluation (in person or via telestroke) within institutional target (e.g., 10–15 minutes of activation)
ED physician/APP	Immediate bedside assessment; coordinates initial stabilization and orders
Primary/bedside RN	Immediate vital signs, weight, IV access ×2, glucose check; accompanies patient to CT
Charge nurse / rapid response (in-hospital activations)	Coordinates resources, notifies stroke team, supports primary RN
CT technologist	Prepares scanner for immediate non-contrast CT ± CTA per Acute Stroke CT Imaging Protocol; patient taken directly to table
Pharmacy	Available for weight-based alteplase dose verification and preparation support per institutional Alteplase Preparation & Administration Protocol
Laboratory	Expedited processing of STAT stroke labs
Stroke Program Coordinator	Notified for concurrent case capture and real-time metric tracking (see institutional Stroke Registry Policy)
Registration (ED activations)	Expedited patient registration without delaying clinical care

6. In-Hospital Stroke Alert — Additional Considerations

- ☐ Bedside nurse/staff member identifying the deficit initiates activation immediately and remains with patient
- ☐ Last known well determined from the most recent documented neuro-intact assessment (e.g., last nursing check, last therapy session) — document the specific time and source
- ☐ Patient's home unit coordinates transport to CT per Stroke Alert priority, bypassing routine transport queue
- ☐ Baseline neurologic status, current medications (especially anticoagulants/antiplatelets), and recent procedures reviewed immediately, as these affect treatment eligibility
- ☐ If the patient is post-operative or has a condition that alters thrombolysis/thrombectomy eligibility, this is communicated to the stroke team immediately, but does not delay initial activation and imaging

7. Stand-Down / Deactivation

- ☐ Stroke Alert stood down by the responding stroke physician/ED physician once imaging and clinical assessment are complete and either: (a) treatment plan is initiated, or (b) an alternative diagnosis (stroke mimic) is identified
- ☐ Reason for stand-down and final diagnosis documented
- ☐ Stand-down does not require reaching a specific diagnosis before deactivating urgency — patient care continues per the identified condition once stroke is excluded or treatment initiated

8. Documentation

- ☐ Time of activation, activation source (EMS/ED/in-hospital), and location
- ☐ Time of stroke team arrival at bedside
- ☐ Last known well / symptom discovery time and source
- ☐ Time of CT initiation and interpretation (per institutional Acute Stroke CT Imaging Protocol)
- ☐ Treatment decision and, if applicable, thrombolysis or thrombectomy times
- ☐ Stand-down time and reason, if applicable

9. Quality Review

- ☐ All Stroke Alert activations logged and reviewed by the Stroke Program Coordinator for timeliness and appropriateness
- ☐ Missed activations (retrospectively identified stroke cases without a Stroke Alert) reviewed at Stroke Committee for process improvement
- ☐ Delays in team response, imaging, or treatment reviewed in a non-punitive quality improvement format, consistent with institutional Stroke Registry Policy performance improvement loop
- ☐ Aggregate activation, response-time, and stand-down data reported at Stroke Committee per defined cadence

Policy Approval

Stroke Medical Director: _____ Date: _____

ED Medical Director: _____ Date: _____

Chief Nursing Officer/Designee: _____ Date: _____

Next Scheduled Policy Review Date: _____

References: AHA/ASA Get With The Guidelines–Stroke program measures; AHA/ASA Target: Stroke initiative; 2019 AHA/ASA Guidelines for the Early Management of Patients With Acute Ischemic Stroke and subsequent focused updates; The Joint Commission Primary/Comprehensive/Thrombectomy-Capable/Acute Stroke Ready certification requirements for stroke team activation and response times; institutional Acute Stroke CT Imaging Protocol, Alteplase Preparation & Administration Protocol, and EMS–Hospital Stroke Collaboration Agreement. This template must be re-verified against the current certifying-body manual and institutional communication technology at the time of adoption.